

PRE-SEND CHECKLIST — the psychiatric instruction

■■ **INTERNAL.** This page never goes to the practice, to the employer, or to the Regulator. Everything else in the pack is written to be produced; this is not.

BEFORE ANYTHING IS SENT

1. Was Review Decision 69983 put in front of him on 12 August 2026? This is now a switch in the build script — set `REVIEW_DECISION_PROVIDED` in `build_instruction_4doc.py` and rebuild. Until it is set, the letter prints a red flag in Part B and cannot be sent by accident.

■ If **no** — set False; the document is not mentioned at all.

■ If **yes** — set True; the letter then carries an express direction not to adopt any finding, characterisation or conclusion in it, and asks whether his opinion would differ if he disregarded it entirely. **Unaddressed, it is the first thing an opposing expert will find.**

2. The fee. Confirm before instructing. The covering email asks for it. The benchmark on file is the PsychGroup indicative range of \$10,000–\$30,000 for a full medico-legal report; ■ **this instruction asks a narrower question than that**, and should be priced accordingly.

3. The practice footer. The report of 13 February 2025 carries “*disclosed for Qsuper and not for medico-legal use*” on every page. ■ **If that appears on this report it is compromised on its face.** The covering email raises it; confirm it has been understood.

4. Insert the date in the instruction where marked.

4a. ■ The commencement date at question 1.3(c). The letter says “*since at least June 2020*”, which is documented by the 8-hour agreement signed 17 June 2020 and cannot be wrong. **If the actual commencement is earlier, insert it** — the longer the period of unimpaired function in this role, the stronger the answer.

5. ■ Confirm the certificate of capacity wording of 3 July 2026. The phrase “*continuation of existing arrangement ... worked and tolerated ... without deterioration*” is quoted twice — in the schedule and at question 3.6. It is the last unverified quotation in the pack. **Read it from the certificate before sending.**

6. ■ Say nothing about which exhibit holds what, or about the span of the production. The letter and the schedule state only what the respondent asserts — a history from 26 October 2022 — and nothing about what the file does or does not contain. **Do not add it back in correspondence.** It is their assertion to prove.

STILL MISSING FROM THE PACK — AND WHY IT MATTERS

7. ■■ The three movement forms — reduced hours approved by the Director on 27.02.2026, 17.04.2026 and 09.06.2026. **Without them, question 3.6 (inherent requirements) cannot be answered safely**, because the role description makes 24/7 shift work mandatory. They are the employer's own documents and they prove the adjustment was granted three times and operated.

8. ■ The April 2025 individual monthly statistics — 269 to 444 calls per shift at about 29% of full time. Without them the clinician cannot address why reduced hours did not reduce intensity, and “*he kept working for fourteen months*” goes unanswered.

9. ■ Who is performing the duties, which shifts, from when — in writing. Currently colleague report only. Load-bearing for 3.6 and for the employer's accommodation question.

SEQUENCING

10. ■■ Serve the AO3 role description on the Regulator as continuing disclosure BEFORE the report is commissioned. She does not hold it — the amended List of Documents has 52 items and no role description of any kind. If the report relies on a document she has never seen, it surfaces through an expert rather than through disclosure. ■ **Serve the document, not its provenance:** no mention of the request for medical information, the exclusion, or 31 July 2026.

11. ■ Assume everything sent to the practice is produced to the Regulator under the notice it holds. That is why the instruction and the covering email carry no strategy, no characterisations, and nothing from the parallel tracks.

■■■ WHEN THE REPORT GOES TO MSH — CITE THEIR OWN POLICY

The adjustments the report specifies are measured against **HR Policy G3 / QH-POL-210, Reasonable Adjustment**, which MSH itself invoked in the request of 31 July 2026. ■ **Its own words, verified:**

*“The question of whether an adjustment is unreasonable or would cause ‘unjustifiable hardship’ is **tested against the whole organisation, not a division or unit within the organisation. The onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable.**”*

⇒ ■■■ **The test is not whether the Logan Switchboard roster can absorb the adjustment. It is whether Metro South Health can — and the onus of showing it cannot is theirs.** The same policy adds that failure to provide reasonable adjustment “*may constitute unlawful discrimination.*”

■ **This goes in the covering note to MSH, not to the clinician.** A psychiatrist does not apply an HR policy. Question 3.6(c) is framed so that his answer — whether adjustments of that kind are ordinarily provided by large employers and by 24-hour health services — maps onto the whole-organisation test without his being asked to apply it.

WHAT THE REPORT MUST NOT DO

■ Say he cannot fulfil the inherent requirements · ■ answer the accommodation question in the negative · ■ opine on working with any named person or reporting line · ■ express any view on whether management action was reasonable · ■ attribute the injury to a course of management conduct generally rather than to the rostering and fatigue strand specifically.